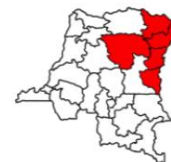




TASK FORCE PRESIDENTIAL EBOLA 17
Ministry of Public Health, Hygiene and Social Welfare
National Institute of Public Health
Public Health Emergency Operations Center
MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°099/MVEBDB/21/08/2026

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HIGHLIGHTS (last 24 hours)

As of August 21, 2026, 83 new confirmed cases of Ebola Virus Disease (EVD), including 27 community cases, were recorded in the provinces of Ituri (62 cases), North Kivu (18 cases), and Haut-Uélé (3). A new health zone has been affected in North Kivu province (Mutwanga).

ACCUMULATIVE CAS 5 458	 ACCUMULATIVE DEATH CONFIRMED 2 606	 LETHALITY 47,7%	PATIENT IN ISOLATION/CTE 798	ACCUMULATIVE HEALED 1 182	TRACKING RATES CONTACTS (Of the Day) 84,7 %
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Affected Provinces and Affected Health Zones



6 Provinces Affected

Uele, Ituri, North Kivu,
South Kivu, Tshopo & Lower Uele



57 out of 151
health zones affected
(37,7%)



Summary of key points from the last 24 hours

In the past 24 hours, **83 new confirmed cases**, including **27 community deaths** from Ebola Virus Disease (EVD), have been reported in the provinces of **Ituri (62 cases)**, **North Kivu (18)** and **Haut Uélé (3)**.

Over the past 24 hours, **15 patients** have been declared recovered after receiving two negative Ebola virus (EV) test results (9 in Ituri, 4 in North Kivu, and 2 in Haut-Uélé). On the other hand, **22 deaths within Ebola treatment centers** have been recorded, including 17 in Ituri, 4 in North Kivu, and 1 in Haut-Uélé.

From the start of the epidemic until August 21, 2026, **5,458 confirmed cases, including 2,606 deaths**, were reported in the DRC, representing a case fatality rate of **47.7 %**. Ituri remains the epicenter of the epidemic, accounting for 83.8% of cumulative confirmed cases and 74.7% of new cases reported in the last 24 hours.

A new health zone has been affected in the province of North Kivu in the last 24 hours, namely that of **Mutwanga**.

Thus, 57 of the 151 health zones in the six provinces have been affected since the start of the epidemic in the DRC. Ituri has 28 health zones affected out of 36, followed by North Kivu (13/34), Haut-Uélé (6/13), Tshopo (7/23), South Kivu (1/34) and Bas-Uélé (2/11).

Distribution of confirmed cases and deaths by affected province					
(As of August 21, 2026)					
Province	Cas confirmed	Death (confirmed)	Lethality	Zones of health affected	New cases confirmed (24h)
Ituri	4572	2 043	44,7%	28/36 (77,8 %)	62
North Kivu	698	476	68,2%	13/34 (38,2 %) 6/13	18
Haut-Uélé	168	77	45,8%	(46,1 %) 7/23	3
Boss	15	8	53,3%	(30,4 %) 1/34 (2,9	0
South Kivu	3	1	33,3%	%) 2/11(18,2%)	0
Bas Uélé	2	1	50,0%		0
Total	5 458	2 606	47,7%	%)	83

Confirmed cases and deaths by province and health zone (situation as of August 21, 2026)							
Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Cas (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	4572	2043	44,7%	62	17	17	34
Give	11	1	9,1%				0
Understand	7	6	85,7%				0
Ariwara	8	2	25,0%				0
Aungba	12	5	41,7%				0
Bamboo	126	28	22,2%				0
have	1	1	100,0%				0
Bunia	1262	395	31,3%	8	4		4
Ladies	27	9	33,3%				0
Drodro	13	6	46,2%				0
Fireworks	73	30	41,1%	1	0		0
Gethy	5	2	40,0%				0
Flounder	2	0	0,0%				0
Kilo	31	12	38,7%				0
Team	57	37	64,9%	3	0		0
Liter	192	106	55,2%	2	0		0
Logo	11	5	45,5%				0
Fight	17	4	23,5%				0
distributed	4	1	25,0%				0
Washing	14	7	50,0%				0
Darkness	30	19	63,3%	5	3		3
Mangala	191	106	55,5%	3	1		1
Mongbwalu	600	283	47,2%	5	1		1
Nanny	190	107	56,3%	5	2		2
Nizi	603	250	41,5%	23	6		6

Legumes	122	35	28,7%	1	0		0
Jungle	10	4	40,0%				0
Rwampara	900	334	37,1%	5	0		0
Tchomia	53	25	47,2%	1	0		0
A valves	THAT	223	THAT	THAT	THAT	17	17
North Kivu	698	476	68,2%	18	9	4	13
Me	108	81	75,0%	1	1	2	3
Butembo	136	103	75,7%	4	2	2	4
Ten	1	0	0,0%				0
Kalunguta	13	6	46,2%				0
Cut	334	222	66,5%	11	5		5
Kyondo	16	12	75,0%	1	0		0
Lubero	1	1	100,0%				0
Be careful.	2	1	50,0%				0
Masereka	7	4	57,1%				0
The museums	68	39	57,4%				0
Mutwanga	1	1	100,0%	1	1		1
Oicha	6	4	66,7%				0
Vuh	5	2	40,0%				0
Haut-Uélé	168	77	45,8%	3	1	1	2
Government	24	9	37,5%				0
Mangbetu							
Gombari	2	1	50,0%				0
Mathemba	63	28	44,4%	3	1		1
Lining	20	15	75,0%			1	1
Beans	1	0	0,0%				0
Ordinary	58	24	41,4%				0
Boss	15	8	53,3%	0	0	0	0
Bafwasende	1	0	0,0%				0
Kabondo	3	1	33,3%				0
Lubunga	1	0	0,0%				0
Maquis-Kisangani	6	4	66,7%				0
Mangobo	2	2	100,0%				0
Boss	1	0	0,0%				0
Wanie-Rukula	1	1	100,0%				0
South Kivu	3	1	33,3%	0	0	0	0
Miti-Murhesa	3	1	33,3%				0
Bas Uélé	2	1	50,0%	0	0	0	0
Blind	1	1	100,0%				0
Viadana	1	0	0,0%				
Total	5458 2606	47,7%		83	27	22	49

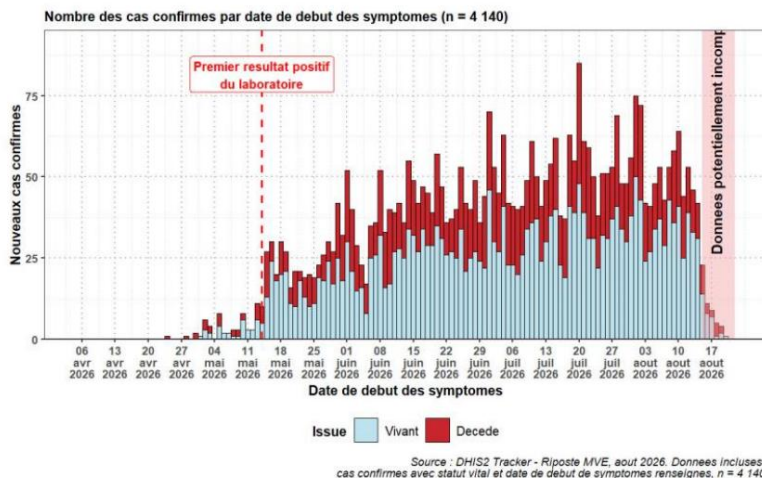
*These are the confirmed deaths that occurred in the various CTEs in the province and are being broken down to be classified by health zone.

NA : Not Applicable.

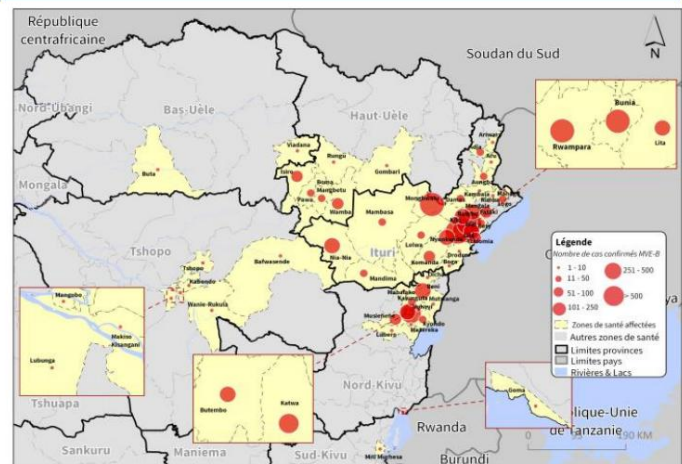
Status of alerts notified by province, as of August 21, 2026

DPS	New Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	39	2	41	16	2	5	0	18	7
Bas Uélé	2	0	2	0	0	2	0	0	0
Ituri	688	75	763	166	77	403	1	243	151
North Kivu	685	40	725	40	34	594	34	74	28
South Kivu	7	0	7	2	0	5	0	2	2
Boss	116	2	118	6	2	99	0	8	3
Total General	1537	119	1 656	230	115	1108	35	345	191

Evolution of cases by date of symptom onset over the last 21 days



MAPPING OF CASES over the last 21 days



1. RESPONSE ACTIONS BY PILLAR

1.1.Coordination

1.1.1. Main Actions

- **Kinshasa:** The Minister of Public Health, Hygiene and Social Welfare received
This Friday, August 21, 2026, at N'djili International Airport, the first doses of Ervebo vaccines, financed by Gavi: 16,520 doses arrived out of a total of 50,120 doses expected.
- **Haut-Uélé:** Joint visit of the security pillars and PoE/PoC in the checkpoints of the city of Isiro; joint visit with the WHO technician to assess the feasibility of installing an isolation tent at the Aero Matari PoC.

- **North Kivu** : arrival of the national Incident Manager and his team via Beni airport, then escorted to the city of Butembo; participation in the joint monitoring and evaluation mission of the response in the health zones of Katwa, Musienene and Butembo; support for the Butembo sub-coordination meeting; harmonization of alert and contact databases and support for the health zone teams.
- **Ituri**: continued evaluation of the technical platform of the CTEs, 15 CTEs having completed their encoding in KoboTools; holding of a cross-border meeting in Arua City (Uganda) on strengthening health security between Uganda and the Democratic Republic of Congo as part of the response to the 17th Ebola outbreak.
- **Tshopo** : participation in the feedback session of the SGI MVE evaluation mission conducted by the central level team; meeting with the chief coordinating physicians and technical and financial partners.
- **Bas-Uélé**: Online meeting between the national level and the logistics pillar and monitoring of response activities; talks initiated with hotels to accommodate high-risk contacts and facilitate their monitoring.

1.1.2. Epidemiological surveillance

1.1.3. Main Actions

At the end of the day on August 21, 2026, 1,656 alerts were recorded, including 1,488 (89.9 % were verified in the six affected provinces. Thus, 345 (23.2%) were validated as suspected cases and all were investigated (100.0%), of which 191 were transferred to CT/CTE.

The proportion of contacts followed up in the last 24 hours has risen to 84.4% (19 829 views out of 23,492 to follow), close to the 85% threshold: Ituri 87.6% (11,937/13,629), Tshopo 99.2% (486/490) and North Kivu 82.8% (6,244/7,545). Haut-Uélé 65.2% (1,042/1,598) and Bas-Uélé 51.3% (120/234).

1.1.4. Challenges

Poor contact tracing around confirmed cases mainly in the provinces of Haut Uélé and Bas Uélé due to insufficient teams dedicated to these activities.

1.2. Update on checkpoint and point of entry (PoC/PoE) activities

1.2.1. Main actions

- Ten (10) alerts were notified to the PoE/PoC in the last 24 hours: five in Ituri (PoC Mankoko 2, AS Mankoko 2, Mambasa Health Zone; PoC Mudzibala, AS Nzere, Bunia Health Zone; PoC Uguro, AS Avere, Mahagi Health Zone; PoC CE 39, AS Nizi, Iga-Barrière Health Zone; PoC Sans Plainte, AS Ombayi, Ariwara Health Zone) and five in North Kivu (PoC Mukulya, Mavivi B and Kasitu, Beni Health Zone). The four active alerts in Ituri were validated after investigation, and three of them were referred to the CT/CTE (75%), with one patient refusing referral; all five alerts in North Kivu were invalidated after further investigation. One (1) lifeless body was intercepted in Ituri (PoC Sans Plainte, ZS of Ariwara) and transported to the Ariwara morgue for swab (100%).

Indicators	Ituri	North Kivu S	South Kivu	Global
Proportion of activated PoCs and Enhanced PoE	100 % 94,4 %	100 % 98,1 %		
Number of people who have switched to PoE/PoC	149 149	85 122	84 432	318 703
% of travelers screened	98,0 % 98,0 %	100 % 98,6 %		

Indicators	Ituri	North Kivu	South Kivu	Global
% of travelers who washed their hands	97,0 % 98,0 %	100 % 98,2 %		
% of travelers made aware	99,4 % 98,0 %	100 % 99,2 %		
Number of alerts notified	5	5	0	10
Number of validated live alerts	4	0	0	4
Number of bodies recovered today	1	0	0	1
Number of problematic contacts intercepted today	0	0	0	0
% of validated live alerts referred to the CT/CTE	75 %	THAT	THAT	75 %
% of lifeless bodies swabed	100 %	THAT	THAT	100 %

1.2.2. Challenges

- Insufficient operationalization of PoE/PoC: six PoCs (Dusu, Apodo, Aungba, PK6 Route Niangara, Gossamu and Magambe) are on strike due to non-payment of service providers; in Ituri, completeness has risen to 100% (60 out of 60 reports) but only 9.09% of strategic PoE/PoCs (1 out of 11) are operating without interruption.

1.3. Laboratory

1.3.1. Main actions

- **Ituri : 62 new positive results** (45 living and 17 deaths) out of 389 new samples received and analyzed (**positivity: 15.9 %**).
- **North Kivu : 18 new positive results** (9 living and 9 dead) out of 114 samples analyzed (**positivity of 15.8 %**).
- **Haut-Uélé: 3 new positive results** (2 living and 1 death) out of 12 samples analyzed (**25% positivity**).
- **South Kivu : 2 new samples** received and analyzed in the Miti-Murhesa health zone, all negatives

1.4. Infection Prevention and Control (IPC/EDS)

1.4.1. Main actions

- **In Ituri**, 21 infection control rings were opened out of 80 confirmed cases (26.0%), 47 households were decontaminated out of 75 identified (62.7%), and all 10 health facilities (100%) were decontaminated. Three distributions of infection prevention and control (IPC) kits were carried out (Bunia and Nia-Nia), and 11 assessments and 49 health facility support sessions were conducted in 8 health zones, where 157 providers were briefed. Of 93 death alerts received, including 13 postponements from the previous day, 60 bodies were collected by swab out of 76 processed (78.9%), and 72 death assessments were performed (77.4%).
- **In North Kivu**, 122 health facilities were monitored and supported, 367 service providers were briefed, 24 handwashing stations were inspected, and one infection prevention and control (IPC) kit was distributed in Katwa; two handwashing stations were made operational in Butsili with the support of

CARITAS Goma; 34 death alerts received, including 16 postponements from the previous day; 33 bodies collected by swab and 38 EDS carried out, for 1 failure linked to resistance in the AS of Kalau (ZS of Beni).

- **In Bas-Uélé**, 10 out of 15 ESSs were accompanied, 11 ESSs remain under PCI surveillance and one ring remains open in Buta and one in Viadana; the HGR of Buta was re-evaluated, its score going from 32% to 61.6%.
- **In Tshopo**, 8 health facilities were supported (Jamaa Health Center, Foyer Health Center, Mangobo General Hospital, La Famille Health Center, Matete Health Center, Segama Health Center, Konga-Konga Health Center, and the Central Prison), where 35 healthcare providers were briefed on the importance of triage and standard precautions; 9 people with disabilities were assessed, including 7 at the ART Polyclinic and 2 at the Tradimoderne Medical Center. Of 3 death alerts and 1 report from the previous day, 1 body was recovered by swab and 1 death report was completed, while 2 alerts were reported.
- **In Haut-Uélé**, 6 infection prevention and control zones (IPCs) were opened (Isiro 3, Boma Mangbetu 2, and Wamba 1) and 6 households were decontaminated; 29 health establishments (HEs) remain under infection prevention and control (IPC) surveillance, 7 HEs have been assessed out of 7 planned, 11 have received support out of 15, and 42 healthcare providers and hygiene workers have been briefed out of 60 expected. Training of providers and hygiene workers on basic IPC has been completed in the Wamba health zone. Of the 6 death alerts received (3 in Isiro and 3 in Wamba), all samples were collected by swab, and 6 blood tests were carried out by the Red Cross team.

1.4.2. Challenges

- Weak implementation of IPC standards: In Ituri, ring openings were limited to 26% due to insufficient decontamination teams (Fataki, Mangala), lack of mobility (Komanda, Bambu), unrecorded addresses, and the refusal of a family in Rwampara (machete attack); 16 swab failures and resistance to EDS in Bambu, Mambasa, Nia-Nia, Fataki, and Mandima, where ECUMER team coverage of health areas remains low. In North Kivu, resistance was reported at the Kalau health center (Beni health zone). In Bas-Uélé, decontamination of the household of relatives of the first confirmed case could not be carried out due to resistance, insufficient IPC supplies, and the base of operations lacked transportation.

1.5. Continuity of care

1.5.1. Main actions

- **In Ituri**, 98 new admissions and 2 readmissions were recorded, with 115 discharges, including 9 recoveries and 14 escapes. Thus, 521 patients are hospitalized in 965 beds, representing an overall occupancy rate of 54%.
- **In North Kivu**, 66 admissions were recorded (5 confirmed and 61 suspected) with 59 discharges, including 4 recoveries at the Katwa Ebola Treatment Center. Thus, 176 patients are hospitalized out of 206 beds, representing an 85.4% occupancy rate.
- **In Tshopo**, a new admission of a suspected case to the CTE HC Kisangani and a discharge of recovery bring to 5 the number of isolated patients (1 confirmed and 4 suspected) for 14 beds, or 35.7% occupancy.
- **In Haut-Uélé**, 11 admissions were recorded, all suspected cases, and 2 recoveries. Thus, 69 Patients are hospitalized in 130 beds, representing 53.1% occupancy.
- **In South Kivu**, 27 suspected cases remain hospitalized.

1.5.2. Challenges

Limited capacity and insufficient technical facilities at some Ebola Treatment Centers (ETCs): the ETCs in Nizi, Kilo, Logo, Mandima, and Kigonze (suspected cases) in Ituri are overcrowded, with 19 patients (5 in Kilo, 3 in Sota, 7 in Lolwa, and 4 in Mandima) receiving no food support, and 14 patients have escaped, including 10 in Sota due to lack of food. In North Kivu, several suspected cases in Butembo, Masereka, Vuhovi, and Kalunguta are isolated in health centers that are not transit centers; there are insufficient nutritionists, and UHT milk is unavailable in the ETCs. In Bas-Uélé, the limited capacity necessitates accelerating the construction of the Buta ETC; water supply is difficult, and no ambulances are available for the response.

1.6. Risk communication and community engagement

1.6.1. Main actions

- **In Ituri**, 184 contacts were identified around confirmed cases (164 in Rwampara across 5 health areas and 20 in Mambasa); 253 people were reached during 12 home visits, 100 during 2 educational talks, 57 during 2 community dialogues, and 1,150 during 11 mass awareness campaigns. Twenty-two radio programs and 6 radio spots were produced and broadcast with 36 participating community radio stations, and 42 IEC (Information, Education, and Communication) materials were distributed.
- **In Tshopo**, two rumors were clarified and resistance lifted on one notified; one recovered patient was assessed and two recovered patients joined the response teams.
- **In North Kivu**, a march in support of the response was organized in Beni with local youth organizations to discourage attacks against the teams; 50 community leaders and influencers in Butembo were briefed to strengthen support for the EDS (Emergency Response Support) program. The discharge of four recovered patients from the Katwa Ebola Treatment Center resulted in the recording of testimonies for local media.

1.1.1. Challenges

- **Persistent community challenges:** In Ituri, there is weak involvement of traditional authorities and leaders in the Mangala (Tchere health zone) and Bunia (Babao Bokoe community) health zones; only 8 of the 28 affected health zones in Ituri and 2 in North Kivu have reported community concerns. Rumors persist about people seeking money in the name of Ebola, water contamination by health workers, and the non-existence of the disease, reported at entry points in Ituri, Tshopo, and Haut-Uélé.

1.7. Logistics

1.7.1. Main actions

- **In Tshopo**, high-visibility vests were provided to service providers of the Entry Points pillar; construction work on the Madula and Kabondo isolation units and the large-capacity CTE continues.
- **In North Kivu**, delivery of nutritional support inputs (plumpy nut) to the Beni health zone and the warehouse was refitted with logistical support from WFP; continuation of the construction of the triage at the Vuhovi General Referral Hospital (ULB Cooperation) and the rehabilitation of the CT at the Musienene General Referral Hospital (MSF France).
- **In Bas-Uélé**, provision of a batch of PCI inputs from FHI 360 (sprayers, vests, glasses, gloves, N95 masks and thermo flash); supply of a second batch of PCI inputs and transport of samples to the Rad-One site in support of BCZ Buta.

1.7.2. Challenges

Persistent logistical needs to support the expansion of the response: completion of the extensions to the Nizi, ISTM, and Bambu treatment centers in Ituri, where 4 ambulances are out of service (Bambu, Bunia, Mongwalu, and Nizi) and where the installation of 4 mobile laboratories is planned (Fataki, Bambu-mine, Lita, and Rwampara); deployment of IPC supplies to the 6 zones in western and northern Bas-Uélé, where basic supplies are on the verge of running out, and no medicines are available .

pre-positioned for supportive treatment and no ambulance is available; in North Kivu, non-payment of June bonus to 60% of logistics providers and no funds allocated for handling during unloading of inputs; in Haut-Uélé, lack of means of transport for supervisions and shortage of inputs, protective equipment and water in all activated PoE/PoCs.

1.8. Security

1.8.1. Main actions

- **In North Kivu**, the pillar escorted the national Incident Manager and his team to Butembo and ensured 7 security escorts, all carried out 100%: coverage of IPC teams in the households of Mangothe and Kasanga Tuha (Bundji AS), at the Ruwenzori CS (Kasabinyole AS) and at the Jades CS (Kanzulinzuli AS); accompaniment of EDS teams to the Bundji cemetery, the Unit Hospital in Mihake and the Maghombi Hospital (Oicha) for the swab, the securing of a body and the evacuation of samples.
- **In Haut-Uélé**, a joint visit of the Security and PoE/PoC pillars was conducted in the checkpoints in the city of Isiro.

1.8.2. Challenges

In Ituri, a response team was chased away with machetes during a decontamination attempt in a household in the Rwampara health zone, and 14 refusals of isolation were recorded in Rwampara, Bunia, Kilo, Damas and Mongbwalu; 14 patients also escaped from Ebola treatment centers, including 10 in Sota due to lack of food.

In North Kivu, resistance prevented the removal of a body in the Kalau AS (Beni ZS).

1.9. PSEA (Prevention of Sexual Exploitation and Abuse)

1.9.1. Main actions

Continued support for training of community, military and police leaders on PSEA and MVE, jointly with the communications pillar.

1.9.2. Challenges

- Still insufficient coverage of PSEA activities, limited to 4 out of 36 health zones in Ituri, lack of mapping of stakeholders, insufficient mass awareness materials and lack of transport and computer equipment for the pillar; Coverage remains limited to 4 of the 36 health zones of Ituri (11.1%) and only two partners (WHO and Africa CDC) report their PSEA activities.

MESSAGES CLES

- Ebola virus transmission remains active, with 83 new cases including 27 community deaths and 22 intra-CTE recorded in the last 24 hours, bringing the national total to 5,458 confirmed cases and 2,606 deaths.
- Ituri remains the epicenter of the epidemic, concentrating 83.8% of cumulative confirmed cases and 74.7% of new cases reported in the last 24 hours and a greater expansion (28 of the 36 health zones affected).
- The monitoring system displays a performance (100%) suspected cases investigated while the follow-up rate (84.7%) is close to the threshold of 85.0%.
- Strengthening priority interventions (surveillance, care, IPC, logistics and community engagement) remains essential to quickly interrupt transmission chains.

TIMELINE OF KEY EVENTS

- From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.
- Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a state of emergency public health measures of national and then continental scope, accompanied by the mobilization of the 64 and support.



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